

The intake that *fills itself*

Due Mon 31 Aug · 11:59 pm IST

Budget 6-10 hours · reply to the email you received this in

— The problem

Patients at a hair and scalp clinic fill a 16-question intake form before their consultation. Today it is paper, typed into software later. People abandon it, answer carelessly, or need a nurse to walk them through it.

The last decade of clinic software was forms and dashboards: the human clicks, the software stores. We want the opposite: the software does the work, the human gets the outcome. Here the outcome is a doctor who has a complete, accurate picture before the patient walks in, and a patient who barely noticed they filled anything.

Build a small web app that gets a patient through this intake. **The only fixed thing is the output:** the form on page 2, fully filled, visible as structured data. Everything else, what the patient sees, hears or taps, is yours. Voice copilot, no AI at all, one screen, many steps: be creative, and design for a real patient in a real clinic.

— What we judge, in this order

1

How it feels

Snappy, obvious without instructions, finishable by a 55-year-old on a phone. We will try it on a phone and a laptop.

2

Taste

How each question gets answered. Some are one tap, some are speech, some should be inferred from an earlier answer and just confirmed. Think per question, not one chat box for everything.

3

Ideas

What you added that we did not ask for, and what you would do with one more week.

4

Resourcefulness

Which models and services you chose and why, what you bought instead of built, how you checked the form actually gets filled correctly, how you deployed. We hire for judgement that reaches a working outcome fast, not for code written from scratch.

Any stack, any paid or free service, any AI coding tool (we use Claude Code every day). We do not judge what you chose; we judge whether you chose well.

— Rules

- Language: Hinglish is fine.
- Build only what the patient touches. No login, no admin panel.
- No real personal data; use made-up patients. Keep API keys out of the repo.

— Submit by replying to the email

1. **A live link** (Vercel, Netlify, Render, anything) that works without us installing anything.
2. **The repo** (GitHub, public or invite `nikhil@thevectorlabs.in`, my GitHub account) with a README: how to run it, your choices (models, services, bought vs built, how you tested the fill), and what you'd improve with one more week.
3. **A 2-minute screen recording** of you walking through it as a patient, naming the two or three decisions you are proudest of. This stands in for a screening call, so treat it as one.

Hair & scalp intake, *16 questions*

A · PERSONAL AND FAMILY HAIR LOSS HISTORY

1. Age when hair loss began (number)
2. Duration · under 6 months / 6-12 months / over a year
3. Family history (multi) · father / mother / siblings with thinning or baldness / no known family history
4. Pattern (multi) · receding hairline / thinning at crown / widening part line / diffuse thinning / patchy loss / sudden excessive shedding

B · HORMONAL AND HEALTH INFLUENCES

5. Diagnosed conditions (multi) · PCOS or PCOD / thyroid disorder / diabetes / autoimmune disease / anemia / none
6. Menstrual cycle · regular / irregular / menopausal / not applicable · female patients only
7. Pregnancy-related hair loss · currently pregnant / postpartum under 1 year / not applicable · female patients only
8. Acne or oily skin in adulthood · yes / no
9. Excess body or facial hair growth · yes / no

C · LIFESTYLE AND ENVIRONMENTAL TRIGGERS

10. In the past 6 months (multi) · crash dieting or major weight loss / high stress or emotional trauma / fever with illness (COVID, dengue, typhoid) / recent surgery / change in location, water or air quality
11. Habits, each yes/no · smoking (if yes: under 5, 5-10, over 10 a day) / alcohol / hard water for hair wash / hair wash frequency (daily, alternate days, weekly) / heating tools or styling chemicals / salon treatments such as keratin, rebonding, smoothening (if yes, which)

D · CURRENT HAIR CARE AND TREATMENTS

12. Products, per row: used yes/no, duration (under 3 months, 3-6, over 6), helped yes/no, side effects yes/no · rows: medicated shampoos / hair oils or serums / topical minoxidil / oral minoxidil / supplements
13. In-clinic procedures, per row: done yes/no, sessions (1-3, 4-6, over 6), helped yes/no · rows: PRP, GFC or iPRF / stem cells or exosomes / hair transplant / other
14. Side effects or poor response to past treatment · yes / no, and if yes, describe

E · SAMPLE AND CONSENT

15. Preferred sample type · saliva / blood / either
16. Consent to sample collection and genetic analysis · yes / no

How you learn the patient's sex for questions 6 and 7, or whether you ask at all, is part of the design. A machine-readable version is at `haikustudio.ai/hiring/intake-schema.json`; field names are not graded, coverage and correctness are.

— After you submit

We review within two working days and reply either way. If it goes well, the next step is a 30-minute call where you walk us through your decisions. Questions: just reply to the email.